



Ryan Schubert <ryan.schubert@gamedaymenshealth.com>

Regional Marketing Fund Allocation Update

Gameday Marketing Team <marketing@gamedaymenshealth.com>

Mon, Jul 27 at 18:07

Bcc: <fa-us@gamedaymenshealth.com>

Hi Team,

Quick recap below on the regional allocation as discussed during the Huddle this morning.

We came to you with an open-ended ask: tell us what would move the needle in your market if you had support from a \$50K regional fund, and why your market needs it. We kept the ask intentionally broad because we wanted your best, most creative thinking, and we wanted a way for HQ to support you beyond what we run at the national level.

We received 70 submissions in total. They ranged from well-built plans with real tracking to local event sponsorships and smaller one-off ideas. A strong subset stood out, and we appreciate everyone who took the time to think about what their market needs and put something forward.

How we evaluated. Every submission was scored against a consistent framework: return potential, reach and impact, trackability and measurability, fit to our core patient, market need and market operational strength. The marketing FAC subcommittee then reviewed the pool together across multiple sessions. This was not a quick pass, and there were some good ideas to weigh.

What became clear. What our markets need most right now isn't more of any single tactic or single local event. It's leads in the door, and new channels to bring those leads in. That insight changed how we thought about the fund. Rather than splitting \$50K across many small, one-off local spends, or putting all of it behind a single market, we decided the highest-value use of this money is to run a disciplined, controlled test of a new lead channel across three markets, in a way that produces a playbook every clinic can eventually use.

What we're doing. We're launching a direct-mail campaign: a brand-new and trackable channel, across three markets. Direct mail lets us reach prospects who don't convert through digital alone, and it gives us a clean, measurable way to attribute leads back to the source. We know some of you have tested mailers locally, but what we haven't done is run one at a true market-wide scale.

On top of the mail drop, we'll build the mailing list into our digital retargeting so the same households get multiple touchpoints, not just one.

We'll be testing a couple of offers and creatives to really understand what works. One set of creative will intentionally speak to women as the household decision-makers, often the person opening and sorting the mail, and frequently the one who nudges the men in their lives to book.

Why these three markets. The common thread isn't performance level. It's operational excellence. For a real test, the dollars and the leads have to land in markets we trust to execute, so the results reflect the channel and not the operations.

We also deliberately chose three markets at different points on the performance curve, so we can see how the same channel performs from different starting lines:

- **Michigan:** one of our stronger markets. The question: can a new channel take an already high-performing market and unlock the next gear of growth?
- **Central Virginia:** a solid market ready for its next phase of growth. The question: can this channel fuel momentum and performance?
- **New England:** a market earlier in its growth curve where clinics need a catalyst. The question: can this channel give them the push to accelerate?

Together, those three give us a true testing ground rather than a single data point.

Shared investment. We'll also be asking each of the three markets to match the fund's investment in their own market. This isn't about cost-sharing for its own sake. It's about making sure the participating clinics are genuinely invested in the program and not just receiving it. Skin in the game from the clinics is what keeps a test disciplined and makes the results something we can all trust.

How we'll measure it. Before anything mails, we'll baseline each clinic on revenue and booked appointments, then measure against that baseline over the next 60 days. If the channel proves out, we'll have both the results and the repeatable playbook to bring it to more of you.

If your market wasn't selected, please hear this. This is not a verdict on your submission. Choosing three test markets meant leaving good ideas on the table this round. This round was designed as a controlled test of a new channel, not a market-by-market grant, and the entire point is that what we learn belongs to the whole network. Your submissions aren't lost. They're informing how we think about lead generation going forward, and there will be more opportunities as we expand what works. If direct mail delivers the way we believe it can, the clinics that weren't part of the test are exactly who benefit next.

For the selected markets / clinics, we'll be in touch with next steps to discuss the shared investment, timelines and expectations.

Thank you all,